PHASE 2 MODULE 6

# **ASSESSMENT & ANALYSIS:** A WHOLE-SYSTEM APPROACH TO UNDERSTANDING MOVEMENT, LOAD, AND CAPACITY.

Rebuilder

# PURPOSE OF THIS MODULE

By the end of this module, you will understand:

- Why we begin with context → not anatomy
- How to identify load mismatches
- How to run the Rebuildr 3-Part Movement Screen
- How to assess capacity, not just strength
- How to summarise findings into a simple blueprint

# THIS IS NOT NEW KNOWLEDGE

It’s a new system for applying it.

Clinicians already know how to assess joints.
The innovation here is:

- the order
- the structure
- the purpose
- the integration
- the delivery model

Rebuildr lets you capture information that simply cannot fit inside a 30/45/60-minute in-clinic appointment.

We are not teaching “new tests.”
We are teaching a system that turns those tests into direction.

# **SECTION 1:**
# CONTEXT BEFORE BIOMECHANICS

# WHY WE START WITH PROGRAM & LIFESTYLE

Anatomy matters — but the reason most active adults get injured is that their daily environment demands more than their current capacity can handle. Before evaluating joints or capacity, we look at:

- the program they follow
- the lifestyle loads they recover from

Because:
You can’t out-rehab poor sleep.
You can’t out-strengthen chronic fatigue.
You can’t out-mobility a sagittal-only program.
You can’t fix pain if the training stimulus keeps causing it.

Before we assess the person, we assess the environment shaping the person.

# **SECTION 1A:** LIFESTYLE LOAD ANALYSIS

# LIFESTYLE LOAD DETERMINES ADAPTATION

Recovery capacity = adaptation capacity.

If sleep, diet, hydration, or stress are poor, tissue capacity drops:

- resilience
- tolerance
- load capacity
- fatigue resistance
- neuromuscular control

Lifestyle load determines how well (or how poorly) the system adapts

# SLEEP

Ask:

- Total hours
- Sleep consistency
- Night-time wake-ups

Encourage:

- Consistent hours and sleep pattern
- Cool, dark room
- 60min screen-free wind-down
- AM sunlight exposure

**Why it matters:**

Poor sleep increases pain sensitivity, decreases motor control, and slows tissue recovery.

# DIET / FUELING

Ask:

- Eating frequency
- Consistent/inconsistent
- Meal timing
- Whole foods

Encourage:

- High protein
- Carbs pre-training
- Whole foods baseline
- Frequent eating

**Why it matters:**

Fuel is recovery. Low fuel → poor capacity → breakdown under load.

# HYDRATION

Ask:

- Daily intake
- Electrolyte use
- High caffeine / low water pattern

Encourage:

- 2–3L/day +
- Start hydration early in day

**Why it matters:**

Hydration supports tissue health, recovery, and load tolerance.

# STRESS LOAD

Ask:

- Workload
- Emotional load
- Cognitive fatigue
- Sleep disturbance

Tips:

- Breathwork
- Walk breaks
- Morning sunlight
- Prioritise work/life balance

**Why it matters:**

Stress alters movement strategy and increases sensitivity under load.

# **SECTION 1B:**
# PROGRAM ANALYSIS

# THE PROGRAM OFTEN IS THE PROBLEM

The errors I see in most porgrams:

- Excess bilateral sagittal training
- Exercise redundancy (3 exercises, same pattern)
- No unilateral exposure
- No progressive overload
- No stability challenge
- No frontal or transverse work
- Inconsistent or spiking volume

Before we rebuild the human,
we rebuild the environment that shaped them.

# WHAT WE ANALYZE IN A PROGRAM

**Movement Biases**

- Bilateral-only
- Sagittal-only
- No rotation
- No lateral loading

**Volume Errors**

- Overload
- Underload
- Sudden spikes
- Every session high intensity

**Fatigue Exposure**

- No long-duration work
- All “fresh reps”
- No tempo
- No conditioning

**ROM Biases**

- Mid-range only
- No deep loading
- Limited overhead patterns

**Stability Gaps**

- No unilateral load
- No anti-rotation
- No tempo-based control

# PROGRAM + LIFESTYLE + ASSESSMENT = THE REAL PICTURE

Assessment shows: **what they can do**
Program shows: **what they actually do**
Lifestyle shows: **what they can recover from**

Together, these three reveal:

- the root cause
- the capacity mismatch
- the compensatory strategies
- the direction of programming

# **SECTION 2:**
# THE REBUILDR 3-STEP WHOLE-SYSTEM ASSESSMENT

# THE FULL-BODY MOVEMENT SCREEN

We assess the **entire kinetic chain** through:

- Joint ROM
- Unilateral Capacity + Stability
- Global Movement Patterns

This captures more info than any 30-60 min appointment ever could

# **STEP 1:**
# JOINT RANGE OF MOTION

# LOWER BODY ROM

We assess:

- Ankle DF → pelvic compensation, asymmetry
- Kneeling knee flexion → quad length, foot/ankle  positioning
- Prone knee flexion → rec fem vs quad length
- Hip flexion → pelvic compensation, non-working leg positioning, asymmetry
- Lumbo-pelvic flexion → hamstring length, pelvic control
- Hip IR → pelvic compensation, asymmetry
- Hip ER (FABER) → pelvic compensation, asymmetry

# UPPER BODY ROM + T-SPINE

We assess:

- Shoulder flexion → rib strategy, thoracic mobility
- Shoulder abduction → scapular rhythm
- Shoulder IR/ER → overhead + pressing capacity
- Thoracic rotation → lumbar compensation, overhead behavior

# UPPER BODY ROM + T-SPINE

We assess:

We are **not chasing numbers.**

We are identifying:

- movement options
- asymmetries
- early compensations
- joint limitations influencing global patterns

# **STEP 2:**
# UNILATERAL CAPACITY & STABILITY

# WHY WE TEST CAPACITY, NOT STRENGTH

Clinics test strength.
Rebuildr tests **capacity**:

Strength = one rep
Capacity = what they can tolerate repeatedly

Because pain in fit, active adults is usually caused by a capacity mismatch, not an isolated strength deficit.

# THE REBUILDR CAPACITY BATTERY

**Lower Body**

- SL calf raise
- Loaded step-up
- Copenhagen hold
- Side plank hip abduction
- Hip-flexor 90° hold

**Upper Body**

- Single-arm DB press
- Quadruped Y-raise hold
- Bottom-up KB hold (90°)
- Plank (core/shoulder capacity)

# UNDERSTANDING CAPACITY

## Capacity is not measured against norms

Most clinicians immediately ask: “What's a normal number for this test?”

Rebuildr is **not** identifying **absolute capacity.**
We are identifying **asymmetrical breakdown.**

Because:

- Pain often comes from capacity mismatches inside the same person.
- Left vs right is more meaningful than person vs population.
- Fatigue strategy tells us more than rep count.

We care about **behavior under load over-time,** not arbitrary standards.

# WE ARE LOOKING FOR DIFFERENCES

## NOT SCORES

Client A
Left step-up: 8 reps
Right step-up: 25 reps

Client B
Left: 20
Right: 22

Client A has a major capacity mismatch.
Client B has adequate symmetry.

**The number doesn’t matter.**
**The difference does.**

# **STEP 3:** GLOBAL MOVEMENT PATTERNS

# GLOBAL PATTERNS REVEAL THE STORY

**1. Squat**

- Load distribution
- Pelvic strategy
- Depth

**2. Hinge**

- Hip vs lumbar strategy
- End-range tolerance

**3. Cossack (Frontal Plane)**

- Ankle/adductor restriction
- Frontal plane control

**4. SL Squat to Bench**

- Unilateral control
- Eccentric control

# **SECTION 3:**
# THE ASSESSMENT SUMMARY

# THE REBUILDR INTERPRETATION PROCESS

## How To Bring It All Together

You've collected:

- lifestyle load
- program biases
- joint mobility options
- capacity baselines
- movement pattern behaviors
- compensations

# Standout Program Gaps

This is always the first filter because environment drives adaptation.
Common patterns I see:

- Excessive bilateral sagittal volume
- No frontal or transverse work
- No rotation
- Mid-range only
- No fatigue exposure
- No unilateral load
- No tempo or control
- No conditioning

I ask:
“What part of this program created the problem the body adapted to?”
That question alone usually reveals the root cause.

# Lifestyle Contributors

I always identify the lowest-hanging fruit.
Common findings:

- inconsistent sleep
- under-fuelled
- dehydration
- high stress + poor baseline conditioning

I ask:
“What recovery weakness is lowering this person’s capacity the most?”

If I improve lifestyle load even 20%, their programming will work 200% better.

# Joint-Level Movement Options

I look for the most influential joints, not small details.
Most common patterns:

- ankle dorsiflexion restriction
- limited hip rotation
- limited hip flexor length
- thoracic rotation limitation
- limited overhead flexion

These joints drive the most global compensations.
I ask:
“What 2-3 joint limitations create the biggest ripple effect up/down the chain?”
Not 15 things — the 2-3 that change everything.

# Capacity Limitations

Most common findings:

- poor hip stability (copenhagen + side plank abduction very limited)
- weak adductors
- poor eccentric control
- asymmetrical unilateral endurance
- poor shoulder stability
- hip flexor endurance very limited
- lumbar/hip compensation for poor deep core control

I ask:
“Where does the system quickly break down, or show significant mismatch L vs R?”

That becomes the focus for Block 1.

# THE 4-PART SUMMARY

After assessments, we clearly identify a clients:

- Global Gaps (programming + lifestyle)
- Joint Limitations
- Capacity baseline
- Compensation strategies

This becomes the programming blueprint.

# WHY THIS FRAMEWORK WORKS

Because it gives you comprehensive data — far beyond what a single 1-hour appointment can capture.

And that data drives:

- Clarity → what's actually contributing to their presentation
- Direction → where to start, what to target, and why
- Confidence → in your assessment, programming, and progressions
- Repeatability → consistent results across every client
- Scalability → grow without sacrificing quality
- Simplicity → a clear structure for decisions and communication

Comprehensive data is the foundation of modern rehab.
This framework gives you all of it — reliably, remotely, and at scale.

# **SECTION 5:**
# HOW THIS WORKS ONLINE

# HOW THIS ASSESSMENT SYSTEM WORKS ONLINE (REBUILDR-READY)

Everything in this module is fully deliverable online inside the Rebuildr platform.

Full operational details are taught in the Rebuildr Workflow Module.

# SUMMARY

Assessment + Analysis = Direction.

We don’t diagnose.

We understand systems.

Once you understand the system,
you know exactly how to rebuild it.